

Delirium & Altered Mental Status — Inpatient Approach

Delirium is acute, fluctuating inattention · it is a symptom, not a diagnosis — find and fix the cause · non-pharmacologic prevention is the evidence base; antipsychotics neither prevent nor shorten it

1 · Diagnose — CAM, and don't miss hypoactive

Delirium = an **acute, fluctuating** disturbance of **attention** and awareness with a medical cause. Screen with the **CAM** (or 4AT):

CAM feature	Required?
1. Acute onset & fluctuating course	Yes
2. Inattention	Yes
3. Disorganized thinking	Either 3
4. Altered level of consciousness	or 4

Diagnosis = features 1 AND 2, plus 3 OR 4. **Hypoactive** delirium (quiet, withdrawn) is the most common and most missed — and has worse outcomes than the agitated 'hyperactive' form everyone notices.

2 · Find the cause — it's a symptom

- Delirium is the brain's response to a systemic insult. Search systematically — mnemonic **DELIRIUM(S)**:
- **D**rugs (benzodiazepines, opioids, anticholinergics, steroids; withdrawal) · **E**lectrolytes/Endocrine · **L**ack of drugs (withdrawal/pain) · **I**nfection · **R**educed sensory input · **I**ntracranial · **U**rinary/fecal retention · **M**etabolic (hypoxia, hypercarbia, glucose, liver/renal) · **S**leep/Subdural
- Targeted work-up by history/exam: meds review, BMP/Ca/Mg, CBC, UA, infection screen, glucose, O2/CO2, ECG; **imaging/LP/EEG only when focal signs, trauma, or no cause is found** — not routinely.

The two most common reversible culprits on the wards: **medications** (especially anticholinergics, benzodiazepines, opioids) and **infection**. Review the med list before ordering a head CT.

3 · Treat — non-pharm first

The evidence-based core is **multicomponent non-pharmacologic** care (the HELP bundle). Antipsychotics do **not** prevent delirium or shorten it (2025 APA).

Lever	Action
Reorient	Clocks, calendars, family, glasses/hearing aids
Mobilize	Out of bed; remove tethers (lines, catheters, restraints)
Sleep	Protect night-time; cluster care; daylight; minimize alarms
De-prescribe	Stop deliriogenic drugs; treat pain, constipation, retention

Reserve low-dose antipsychotics for severe agitation that threatens safety or essential care, after non-drug measures fail — as a short bridge, not a treatment. Avoid in Parkinson's/Lewy body; benzodiazepines only for alcohol/sedative withdrawal.

Prevent & protect

- Highest yield is **prevention** in at-risk patients (older, baseline cognitive impairment, sensory deficits, severe illness): the same HELP levers cut incident delirium.
- Avoid physical restraints and indwelling catheters — they worsen and prolong delirium.
- Distinguish from **dementia** (chronic, no inattention) and **depression**; delirium fluctuates and impairs attention.

Do/Avoid

- **DO** screen actively with CAM/4AT — especially for hypoactive delirium
- **DO** treat delirium as a symptom and hunt the cause (meds + infection first)
- **DO** lead with multicomponent non-pharmacologic care and de-prescribing
- **AVOID** antipsychotics to prevent delirium or 'treat' the diagnosis — they do neither
- **AVOID** benzodiazepines except for alcohol/sedative withdrawal (they cause delirium)
- **AVOID** restraints and unnecessary catheters/lines